

BY RAMAN SANKAR

THE SMOKE BEHIND THE BAN

Riyan Parag's IPL vape episode triggers a larger debate over whether prohibition curbed vaping in India or merely pushed a fast-growing market dangerously underground

When cameras caught Riyan Parag—captain of Rajasthan Royals—vaping inside the dressing room during an Indian Premier League (IPL) match against Punjab Kings last week, the internet erupted. The Board of Control for Cricket in India (BCCI) fined him 25 per cent of his match fee, handed him a demerit point for "conduct that brings the game into disrepute" and issued a statement about exploring "stringent action". Then, within a news cycle, the conversation moved on.

It shouldn't have. Parag's vape points to a far more serious public health challenge. The question India should be asking is not whether a cricketer behaved badly. It is: why is a product that is completely banned in this country so casually, so visibly, available?

India enacted the Prohibition of Electronic Cigarettes Act (PECA) in 2019—one of the most comprehensive vaping laws in the world, covering manufacture, import, sale, storage and advertisement. First-time offenders face up to a year in prison. Yet hundreds of Indian websites sell them openly. Grey markets persist across cities and small towns. In 2025, authorities seized vaping products worth some $1 million, or Rs 10 crore.

The difficulty with a blanket ban on any popular product is familiar: it does not eliminate demand, it redirects it—in doing so, it leaves behind consumer protections that only a regulatory framework can provide. There is an economic dimension worth noting too. India's e-cigarette market—effectively entirely grey given the ban, but estimated at $2.2 billion, or Rs 21,178 crore—is outside the tax net, generating no revenue and no legitimate domestic industry. Countries that regulate, rather than ban, can levy duties and enforce standards.

When people debate vaping, the conversation tends to get stuck on nicotine—addictive, harmful to developing brains and a legitimate concern. But it may be the least of our worries. E-cigarettes heat a liquid to produce an aerosol whose full chemical implications science is still mapping. Vapours contain formaldehyde, acetaldehyde and heavy metals including lead, nickel, chromium and cobalt—many leaching from the metallic coils that heat the liquid. Researchers link chronic vaping to lung injury, Chronic Obstructive Pulmonary Disease, cardiovascular stress and, in extreme cases, bronchiolitis obliterans or 'popcorn lung'.



Illustration by RAJ VERMA

INDIA CAN LEARN FROM THE EU: THEY MANDATE MANUFACTURERS TO CAP NICOTINE CONCENTRATIONS, CUT OUT HARMFUL ADDITIVES AND PUT A HEALTH WARNING ON PACKAGING

In India, because no regulated market exists, there is no testing regime, no ingredient disclosure and no quality standard. Products circulating through grey channels could contain diacetyl, vitamin E acetate or heavy metals from cheap coils. The experience of Europe offers a useful reference point. Under the European Union's Tobacco Products Directive (TPD), enforceable since 2016, manufacturers must submit toxicological dossiers on every ingredient, nicotine concentrations are capped, packaging carries mandatory health warnings and products must be child-resistant. Crucially, harmful additives including diacetyl are prohibited from compliant e-cigarette liquids under the TPD's ingredient safety requirements, with several member states going further still.

The policy response requires two parallel moves. First, a stronger enforcement of the existing ban. The persistence of open online and offline sales and social media advertising suggests that customs authorities, state police and digital regulators would benefit from greater coordination around the grey market. Second, India needs a serious conversation about a regulatory framework—one that does not abandon the protective intent of PECA but is honest about the limits of prohibition alone. This is not a concession to the vaping industry—it is a basic act of consumer protection. Right now, a youth in India has no way of knowing what they are inhaling.

The camera caught one young man vaping in a dressing room. It opens the door to a much larger and more important conversation: about a growing grey market, unknown chemical risks and what it would take to ensure that consumers—whatever their choices—are not left completely unprotected. That conversation is overdue. ■

Sankar is a public health and policy professional